MODULE
5

Bladder & Kidney

The Posterior Circuit Closes — The Largest Channel of the Body

Fourth channel pair of the 12 Primary Meridians — 67 and 27 points

This Module Covers:

- The Bladder Meridian of Foot-Taiyang (BL) — 5 branches, 67 points, the largest so far
- The Kidney Meridian of Foot-Shaoyin (KI) — 3 branches, 27 points
- How BL and KI close the Posterior Circuit and hand off to Pericardium (PC)
- Clinical context from the transcript: Back-Shu points, heel pain, SP6
- Ballroom & DanceSport relevance

Sourced from the 2026 Lecture 5 deck (Dr. Zhang, 44 slides) and the Week 5 class transcript. Like Module 4, this draws on the newer 2026-series deck rather than an older Week_N deck, per the project's established source hierarchy.

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How to Use This Module

BL and KI are the seventh and eighth of the 12 primary meridians, closing the Posterior Circuit (Module 1, Section 6: HT to SI to BL to KI) before Qi hands off into the Middle Circuit at the Pericardium Meridian. BL is the largest channel in the entire course — 67 points across 5 branches — so this module leans on the branch structure even harder than Module 3 did for the Stomach Meridian.

1. The Bladder Meridian of Foot-Taiyang (BL)

Meridian Clock: 3:00–5:00 p.m. Part of the Posterior Circuit, paired as Fu with the Kidney (Zang). The transcript is direct about why this channel is so large: it connects internally with more organs than any other — lung, liver, heart, and the kidney itself — via the Back-Shu points that run down its third branch, one point per organ. Back-Shu points themselves are deferred to the future Special Points module, but the concept — that this channel's back-line points are organ-specific access points — is worth knowing now.

Running Course — Five Branches

1. Facial/vertex branch: from the inner canthus (Jingming, BL1), ascends the forehead, joins the Governor Vessel at the vertex (Baihui, GV20).
2. The temple branch: from the vertex, runs around the temple through 6 named crossing points (GB7–GB12).
3. The straight portion: from the vertex, enters and bifurcates around the brain, descends the posterior neck, runs parallel to the spine down the medial scapular region (1.5 cun lateral to the spine) through the lumbar region, then goes internally to connect with the kidney and pertain to the bladder.
4. The first back branch: from the lumbar region, descends through the gluteal region to the popliteal fossa.
5. The second (largest) back branch: from the posterior neck, runs the lateral pathway (3 cun from the spine, through the medial border of the scapula), down through the gluteal region, thigh, and into the popliteal fossa — meeting the first back branch there — then continues down the calf, behind the external malleolus, along the 5th metatarsal, ending at the lateral tip of the little toe (Zhiyin, BL67), linking with the Kidney Meridian.

> *The two back branches run parallel to the spine at two fixed distances, both given directly in the lecture: the medial pathway is 1.5 cun from the spine (Back-Shu points, BL11–BL30), and the lateral pathway is 3 cun from the spine (BL41–BL54). The transcript adds a hand-measurement trick for the 1.5 cun distance: held together, the four fingers span roughly 3 cun, so half that width — two fingers — approximates 1.5 cun.*

First and Last Points

	Point	Category
First point	Jingming (BL1)	—
Last point	Zhiyin (BL67)	Jing-Well Point

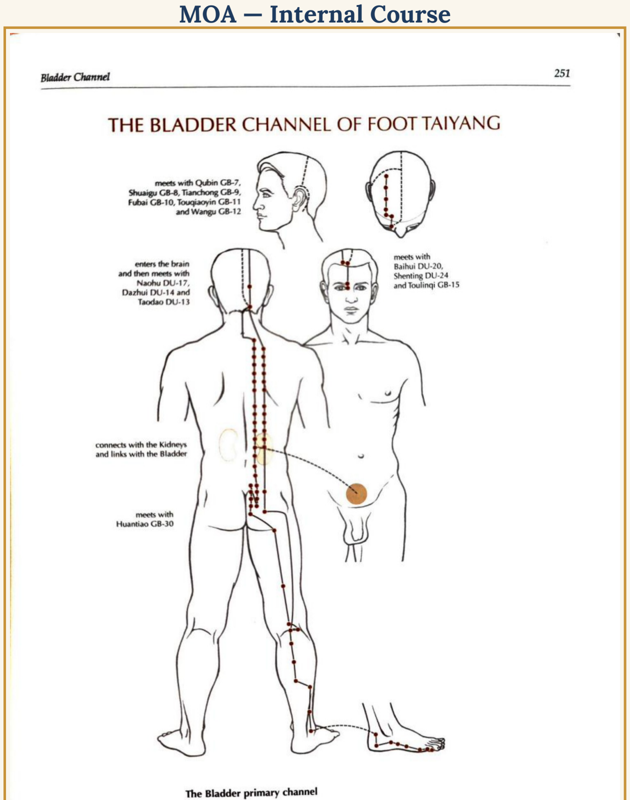
> *Safety note from the transcript, not the slides: Dr. Zhang explicitly advises against needling the points immediately around the eye (BL1 and neighbors) early in training, given how close they sit to the orbit — stated directly as a caution, not a general point property.*

Crossing Points (14 points)

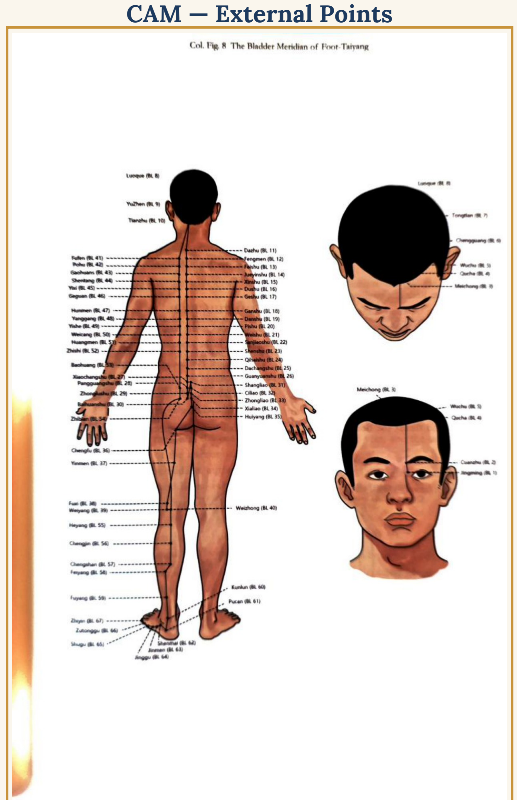
The transcript confirms this directly: “it has 14 crossing points, belonging to two meridians — one is the Governor meridian, the other is the Gallbladder meridian.” The full list, with locations where the source gives them:

Point	Location
Taodao (GV13)	Below the T1 spinous process
Dazhui (GV14)	Below the C7 spinous process, roughly level with the shoulders
Fengfu (GV16)	1 cun above the midpoint of the posterior hairline
Naohu (GV17)	2.5 cun above the midpoint of the posterior hairline
Baihui (GV20)	Midline of the head, 5 cun above the anterior hairline
Shenting (GV24)	0.5 cun above the midpoint of the anterior hairline
Qubin (GB7)	On the head, where the temple's posterior border crosses the ear-apex line
Shuaigu (GB8)	Above the ear apex, 1.5 cun within the hairline
Tianchong (GB9)	Above the posterior ear border, 2 cun within the hairline
Fubai (GB10)	Posterior/superior to the mastoid process
Touqiaoyin (GB11)	Posterior/superior to the mastoid process, below Fubai
Wangu (GB12)	Posterior/inferior to the mastoid process
Toulinqi (GB15)	Above the pupil, 0.5 cun above the anterior hairline
Huantiao (GB30)	Between the greater trochanter and the sacral hiatus

MOA — Internal Course



CAM — External Points



Left: internal course and crossing points (MOA). Right: all 67 external points, Jingming (BL1) to Zhiyin (BL67) (CAM).

Summary of the Bladder Meridian

- Pertaining organ: Bladder. Connecting organ: Kidney.
- Running course: head to foot. The largest channel in the course — 67 points across 5 branches.

Main Syndromes & Indications

A. External course symptoms

- Alternating chills and fever, headache, stiff neck.
- Pain in the lumbar region, nasal congestion, diseases of the eye.
- Pain along the back of the leg and foot.

B. Internal organ (Bladder) symptoms

- Pain in the lower abdomen.
- Retention of urine; painful urination.

> ***Ballroom/Dance Relevance — the entire posterior chain, named as a single channel (original synthesis, not lecture content)***
>
> BL is the clearest single-channel argument for why 'posterior chain' matters as a trained concept in dance, not just a gym phrase: one continuous meridian runs the full erector/hamstring/calf line from the eye to the little toe. The two fixed back-branch distances (1.5 and 3 cun from the spine) roughly bracket the paraspinal and erector muscle groups that fail first when frame collapses under fatigue — already flagged in Module 1's Posterior Circuit sidebar, now with the specific channel and its actual point-line behind that claim.

2. The Kidney Meridian of Foot-Shaoyin (KI)

Meridian Clock: 5:00–7:00 p.m. Part of the Posterior Circuit, paired as Zang with the Bladder (Fu). Closes the Posterior Circuit, handing off to the Pericardium Meridian — the first channel of the Middle Circuit.

Running Course — Main Course Plus Two Branches

1. Main course: starts beneath the small toe, crosses the sole (Yongquan, KI1), emerges below the navicular tuberosity (KI2), runs behind the medial malleolus (KI3), the heel (KI4–KI6), the medial leg (KI7–KI9, crossing Sanyinjiao SP6), the popliteal fossa (KI10), the thigh, and the vertebral column (meeting Changqiang, GV1) — then internally pertains to the kidney and connects with the bladder.
2. The straight portion: re-emerges from the kidney, ascends through the liver and diaphragm, enters the lung, runs the throat, and ends at the root of the tongue. The transcript notes the Spleen Meridian also connects to the tongue root (Module 3) — tongue diagnosis distinguishes Spleen function at the lower/front surface from Kidney function at the root specifically.
3. The branch: from the lung, connects with the heart and flows into the chest, linking with the Pericardium Meridian of Hand-Jueyin — the Middle Circuit's first channel.

> *Clinical pearl from the transcript, not the slides: heel pain is specifically named as a first-line indicator to consider Kidney function, since the main course runs directly behind the heel. Also noted: KI1 (Yongquan) is described as commonly pressed/massaged rather than needled, given how painful direct needle stimulation is at that location.*

First and Last Points

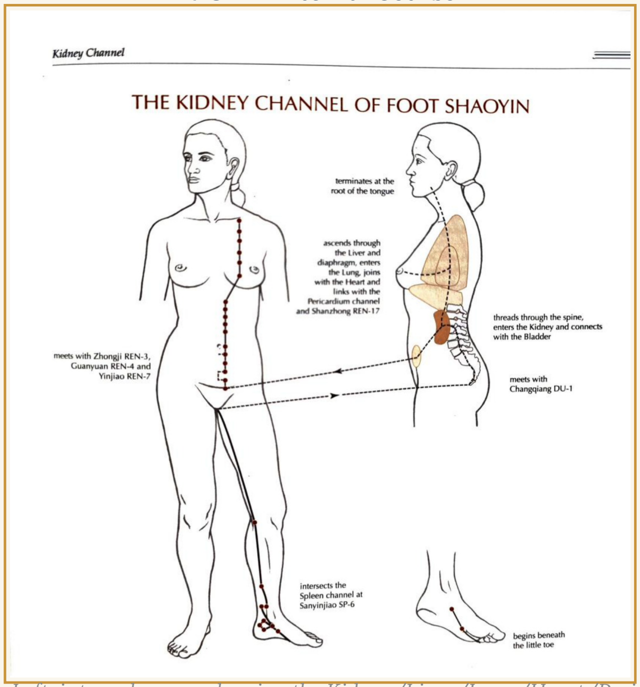
	Point	Category
First point	Yongquan (KI1)	Jing-Well Point
Last point	Shufu (KI27)	—

Crossing Points

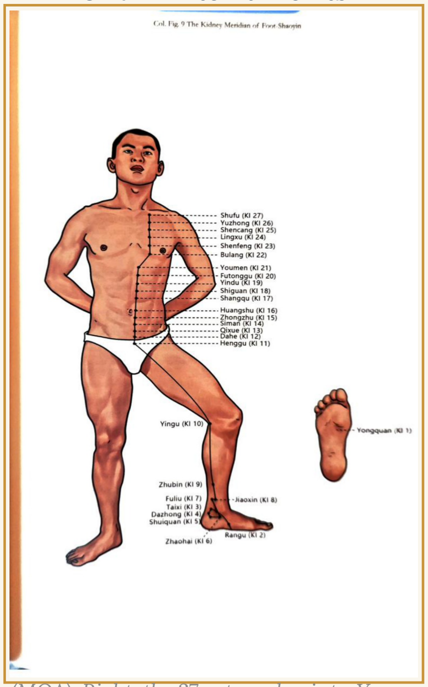
Point	Location
Zhongji (CV3)	Anterior midline, 4 cun below the umbilicus
Guanyuan (CV4)	Anterior midline, 3 cun below the umbilicus
Changqiang (GV1)	Midway between the coccyx tip and the anus
Sanyinjiao (SP6)	3 cun above the tip of the medial malleolus

> *Sanyinjiao (SP6) gets specific clinical emphasis in the transcript beyond its role as a crossing point: described as a primary point for reproductive/gynecological complaints and menstrual pain, matching its name ("three Yin meridians meeting" — Spleen, Liver, and Kidney all cross here). Worth remembering for the capstone, given the female athlete population in DanceSport.*

MOA — Internal Course



CAM — External Points



Left: internal course showing the Kidney/Liver/Lung/Heart/Pericardium connections (MOA). Right: the 27 external points, Yongquan (KI1) to Shufu (KI27) (CAM).

Summary of the Kidney Meridian

- Pertaining organ: Kidney. Connecting organ: Bladder.
- Running course: foot to chest, linking with the Pericardium Meridian of Hand-Jueyin.
- Total points: 27.

Main Syndromes & Indications

A. External course symptoms

- Pain along the lower vertebrae; coldness in the feet.
- Motor impairment or muscular atrophy of the foot.
- Dryness in the mouth, sore throat.
- Pain in the sole of the foot or along the posterior lower leg/thigh.

B. Internal organ (Kidney) symptoms

- Ashen complexion, blurred vision, shortness of breath.
- Frequent urination, incomplete voiding.
- Chronic diarrhea or constipation, abdominal distention, insomnia.

> ***Ballroom/Dance Relevance — endurance reserve and the fatigue that isn't muscular (original synthesis, not lecture content)***
>
> KI's internal symptom list — shortness of breath, insomnia, an ashen look — reads as genuine overtraining/under-recovery territory, distinct from BL's more mechanical posterior-chain-under-load picture. Worth holding the two channels as separate diagnostic directions for the same complaint ('I'm exhausted'): BL points toward mechanical/postural fatigue, KI toward deeper reserve depletion. SP6 (Section 2) is also worth flagging here for female competitive dancers specifically, given the transcript's emphasis on its reproductive/menstrual applications.

3. Closing the Posterior Circuit

HT to SI to BL to KI is now complete — the full Posterior Circuit from Module 1, Section 6. KI's third branch (lung to heart to Pericardium) is the actual handoff into the Middle Circuit, the next channel being the Pericardium Meridian of Hand-Jueyin, covered in Module 6.

4. Ballroom & DanceSport Relevance — Summary

Original synthesis, not lecture content — flagged as such throughout, consistent with Modules 1–4.

- BL and KI complete the Posterior Circuit's dance-relevant territory: BL as the mechanical posterior-chain line, KI as the deeper endurance/recovery signal.
- Between the eight channels covered so far (the full Anterior and Posterior Circuits), a genuine pattern is forming: each channel pair distinguishes a mechanical/structural complaint from a systemic/organ-level one along the same body line. This structure itself may be worth proposing directly for the capstone's injury-framework organization once the Middle Circuit is complete.

5. Still Open / Not Yet Sourced

- Back-Shu points — named and located conceptually (Section 1) but not detailed point-by-point; still deferred to the future Special Points module alongside Yuan-Source, Luo-Connecting, Xi-Cleft, and Eight Confluent Points.
- Finger measurement's specific cun ratios — still unconfirmed, carried over from Module 2.
- The exact abdominal/chest channel spacing relative to the midline — still unconfirmed, carried over from Module 3.

Source Notes

Primary source: 2026Lecture_5Vivian.pdf (44 slides, Dr. Zhang, Lecture 5) and AC300Week5BLKD.txt (class transcript). MOA and CAM figures are the same reference figures used across the weekly study kits. Section 4 is flagged separately as original synthesis. The transcript also includes clinical/research color from Dr. Zhang's own practice specialty (reproductive acupuncture) — omitted here as outside this module's channel-theory scope, except where it bears directly on a point's clinical application (SP6).